PathwayAI | Hudson Valley

Dutchess County Lyme Burden & Action Brief

Public evidence for a practical support discussion · Version 1.1

1. OBSERVED PUBLIC EVIDENCE

Area	Reported cases · 2024	Reported rate per 100,000
Dutchess	1,060	355.5
Orange	964	237
New York State	21,632	109.8

County of residence, not exposure location. Official published rate; exact population denominator and vintage not independently verified.

2. STUDY-BASED ECONOMIC ILLUSTRATION

Calculation · original 2016 dollars	Illustrative amount
1,060 reported cases × USD 2,032	USD 2,153,920

Hook et al. (2022): 613 participants in societal analysis; reported cases recruited in four high-incidence states during 2014–2016. Societal: medical, nonmedical and productivity costs; components are already included.

Applying this study mean to reported county cases is an assumption; the result is not measured local spending. No adjustment for underreporting or local case mix. No causal savings or disability probabilities are inferred.

LOCAL RESOURCE AND LANGUAGE CONTEXT

General healthcare resources · 2023	Count
Primary-care physicians	233
Hospitals	2
Hospital beds	404

HRSA AHRF resource counts do not establish Lyme expertise, current staffing or available appointments. Existing HRSA AHRF 2024–2025 asset; 2023 general resource counts, not appointments or Lyme expertise.

Language other than English at home: 16.2% of residents age 5+, ACS 2020–2024. This is not a measure of English proficiency or interpreter demand.

3. ADDITIONAL BURDEN TO MEASURE

Work changes, unpaid care, travel and daily-life limitations matter. The selected study population does not represent every local household. Continuing costs, Lyme-attributable disability and local unmet support needs remain unmeasured here.

From evidence to a support question

4. SUPPORT DECISION TO EXPLORE

Care without insurance and medical bills

Question: Could existing services meet reported uninsured-care needs, or would an additional funded response be needed?

Possible response: Review health-center sliding-fee access, coverage enrollment help and hospital financial assistance with patients; identify staffing, appointment capacity, costs and a possible funding source before proposing new services.

Potential partners to approach: Patients, health centers, hospital financial-assistance staff, enrollment assisters, county planners and potential funders

Proposed actions for local review—not findings about demand, committed partners or a funded service.

5. CHECK FEASIBILITY BEFORE A PILOT

- Confirm local demand with residents and caregivers, including assisted/offline participation.
- Verify current services, eligibility, interpreter access and appointment capacity.
- Agree who is responsible and estimate staff time, program costs and a funding source.
- Define consent, minimal data collection, access, retention and deletion before any collection.

6. AGREE ON FOLLOW-UP

Candidate measures: Care appointments reached; enrollment assistance completed; financial-assistance decisions; remaining bills and unmet needs over a common period

Agree a common reporting period and baseline, record missing answers, measure support received and unresolved needs. Before/after changes alone do not establish causality or savings.

PUBLISHED AND PATIENT-REPORTED BURDEN

- Work changes, unpaid caregiving, travel and daily-life limitations can affect households beyond medical bills.
- Hook societal costs already contain some of these components; do not add them again.
- Horn (2025) describes selected three-month follow-up participants, not county disability risk. Johnson (2014) describes a selected persistent-illness survey, not all new Lyme cases.
- The project’s voluntary 33-person survey snapshot is across locations; question-level denominators remain unverified. No survey percentages or dollar amounts are extrapolated to this county.

SOURCES AND METHODS

1. Reported cases, county of residence · 2024 · Open source
2. Reported rates per 100,000 · 2024 · Open source
3. Societal study mean · 2016 dollars; recruitment 2014–2016 · Open source
4. General healthcare resources · 2023 fields; AHRF 2024–2025 release · Open source
5. Language spoken at home, age 5+ · ACS 2020–2024 · Open source
6. Published follow-up context · Horn et al. 2025 · Open source
7. Published work and function context · Johnson et al. 2014; 2013 survey · Open source

The source ledger and CSV retain source URLs, review status, periods, assumptions and inflation provenance. Official source pages have not been freshly retrieved in this review.

Public evidence and proposed planning actions only. No session stories, personal records or synthetic patient cards are included.

Public-data demonstration. No county or organizational endorsement is implied.